



Health System Management Assignment-II

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Health Sector Development Program-III

The Policy Framework

1.Introduction

Health care is one of the crucial components of basic social services that has a direct linkage to the growth and development of a country as well as to the welfare of a society. Effective planning and implementation of health services requires mobilization of the collective efforts of the relevant national and international organizations. One of the mechanisms to ensure this collaboration is harmonization of health policies, strategies and implementation modalities with national and global perspectives to create synergy and to enhance the efficiency and effectiveness of health service delivery. As has clearly been articulated in the various health sector documents, both HSDP-I and HSDP-II were sector-wide approaches implemented within the context of these national and international policy environments. This section, therefore, deals with the national and global policy contexts that are taken into account during the formulation of HSDP-III. The formulation of HSDP-III will be a five-year programme intended to be implemented from 2005/06 to 2009/10 (July 1997 EFY to June 2002 EFY). The main policy contexts that are considered during the design and implementation of HSDP-III are a commitment towards the achievement of MDGs by aligning HSDP-III with the PASDEP; institutionalization of community health services through the implementation of HSEP (rural, urban, pastoralist and school); the conjoint Accelerated Expansion of PHC Services; implementation of Health Human Resource Development Plan, the Health Care Financing Strategy, Essential Health Service Package, Child Survival Strategy, the National Reproductive Health Strategy, and the National Health Communication Strategy. Other policies that have important bearing on the Health Sector Development are the National Drug Policy, National HIV/AIDS Policy, National Women's Policy and National Population Policy. Moreover, the ongoing political and administrative reforms and strategies such as Agricultural Development Led Industrialization (ADLI), Rural Development Policy and Strategy, Policy on Decentralization, and Capacity Building Program are very important policy initiatives.

2.Vision of HSDP

To see Healthy, productive and prosperous Ethiopians

3.Mission of HSDP

To reduce morbidity, mortality and disability, and improve the health status of the Ethiopian people through providing a comprehensive package of preventive, promotive, rehabilitative and basic curative health services via a decentralized and democratized health system in collaboration with all stakeholders.

4.Values of HSDP

The following are selected as values and philosophies of the sector after a deeper analysis of the internal and external contexts and mandates of the sector. They are believed to provide commitment and momentum to the efforts made to effectively discharge the responsibilities of the sector and meet the expectations of the public.

1. Focus on promotive, preventive and basic curative aspects of health care

2. Deliver integrated, efficient, quality, equitable and pro-poor health service
3. Efficient use of resources and application of appropriate technology
4. Involve the community on health care decision-making process
5. Promote transparent, result oriented and democratic working culture
6. Abide by professional ethics
7. Sense of urgency for the national development
8. Enhance teamwork, partnership and multisectoral approach
9. Be gender sensitive 10. Be ready for continuous change

5.Goals, Objectives, Strategies , Targets and Key components of HSDP-III

The following sections describe the goals, major objectives, and specific objectives of the HSDP components and key activities.

5.1.Goals of HSDP

The ultimate goal of HSDP-III is to improve the health status of the Ethiopian peoples through provision of adequate and optimum quality of promotive, preventive, basic curative and rehabilitative health services to all segments of the population. The following are the general goals of HSDP-III that contribute to the achievement of the ultimate goal:

1. to reduce child mortality;
2. to improve maternal health;
3. to combat HIV/AIDS, malaria, TB and other diseases.

5.2. The major objectives of HSDP-III

- 1) cover all rural kebeles with HSEP to achieve universal primary health care coverage by year 2008;
- 2) to reduce U5 mortality rate from 123 to 85 per 1000 population and infant mortality rate from 77 to 45 per 1000 population;
- 3) to reduce maternal mortality ratio to 600 per 100,000 live births from 871;
- 4) to reduce total fertility rate from 5.9 to 4;
- 5) to reduce the adult incidence of HIV from 0.68 to 0.65 and maintain the prevalence of HIV at 3.5;
- 6) to reduce morbidity attributed to malaria from 22% to 10%;
- 7) to reduce case fatality rate of malaria in age groups 5 years and above from 4.5% to 2% and case fatality rate in under-5 children from 5% to 2%; and
- 8) to reduce mortality attributed to TB from 7% to 4% of all treated cases.

5.3.Strategies of HSDP-III

In order to achieve the aforementioned goals and objectives, HSDP-III has adopted the following strategies:-

- Vigorous implementation of the Health Service Extension Program for the effective prevention and control of communicable disease and promotion of healthful living;
- Improving the quality of health care through provision of adequate resources, implementation of a two way referral system, enhancing the capacity of HEWs for the detection, referral and follow-up of patients, and strengthen secondary and tertiary hospitals, and referral laboratories;
- Improving the number, skills, distribution and management of health workers; ensuring the planned training of health managers in adequate number and appropriate knowledge and skills;
- Mobilize adequate financial resources, ensure efficient utilization, and strengthen sustainable financing mechanisms for the health sector;
- Improving the health information system and the capacity for effective monitoring and evaluation;
- Improving the logistic management system.
- Ensure full community participation in the planning, implementation, monitoring and evaluation of health care;
- Promoting intersect oral collaboration, harmonization and alignment, and coordinate the activities of the public sector, private sector, international organizations and NGOs in health interventions.

5.4.Components and Targets of HSDP-III

HSDP has been divided into seven components in order to facilitate the planning and budgeting process. These are:

1. Health Service Delivery and Quality of Care: - This subcomponent focuses on strengthening the preventive, curative and promotive aspects of health care mainly through the implementation of HSEP. Maternal and child health services, HIV/AIDS, Malaria, Tuberculosis and personal and environmental hygiene will also be given due attention. Accordingly, DPT3 coverage is expected to increase from 70% to 80%, contraceptive prevalence from 25% to 60% and the proportion of deliveries attended by skilled health workers from 9% to 32% during the programme period. In addition by the end of the programme period, VCT services will be provided in 100% of Hospitals & Health Centers and PMTCT service at 100% of the Hospitals and 70% of the Health Centers. It is also planned to increase the number of People Living with HIV/AIDS (PLWHA) on Antiretroviral Therapy (ART) from 13,000 to 263,000; the proportion of households utilizing 2 bed nets from 2% to 100%; and tuberculosis treatment success rate for smear positives from 76% to 85%.

2. Health Facility Construction, Expansion, Equipping and Access: - The component aims at increasing access and improving the quality of health services through the rehabilitation of existing health facilities and construction of new ones and provision of the necessary inputs such as medical equipment and furniture. Thus, the potential health services coverage will be increased from 72% to 100 % during the programme period through construction of new health facilities and upgrading of the existing ones. 80% of the health facilities will also be equipped and furnished as per the standard; and 30% of the Health Centers will be upgraded to enable them to provide Emergency Obstetric Care Service. xiv Draft HSDP-III

3. Human Resource Development: - The HRD component aims at training and deploying of relevant and qualified health workers of different categories for the whole sector and improving the management of human resource within the public sector in order to enhance the efficiency of the health workers, retaining them within the sector and maintain a high level of professional ethics. Hence, all health facilities will be staffed according to their respective standards and Regional Health Bureaux (RHBs) and District Health Offices (DHO) as per the demand of the organizational structure. Through these intervention, the ratio of HEWs to population is expected to reach 1:2,500; and the ratio of midwives to women of reproductive age group will increase from 1:13,388 to 1:6,759. Health Human Resource Development Strategy will be developed and implemented in order to address both the production and retention of staff in the sector.

4. Pharmaceutical Service: A well functioning pharmaceutical service is the corner stone for any worthwhile health service. Thus, this intervention aims at ensuring regular and adequate supply of effective, safe and affordable essential drugs, medical supplies and equipment in the public and the private sector and ensuring their rational use. A comprehensive Logistic Master Plan will be developed and implemented during HSDP-III. The plan is to increase availability of essential drugs from 75% to 100% in all public health facilities; to scale up the supply of imported and locally produced drugs; increase safety, efficacy and quality investigation from the present 40% to 100 %; and to reduce the overall drug wastage from 8% to 1%.

5. Information, Education and Communication:- This component aims at improving the Knowledge, Attitude and Practice (KAP) on personal and environmental hygiene and common illnesses and their causes; and promotion of political and community support for preventive and promotive health services through educating and influencing planners, policy makers, managers, women groups and potential end users . xv Draft HSDP-III Hence, the main targets are provision of appropriate health communication materials to 100% of the HEWs and equipping 100% of the Kebeles implementing HSEP with portable IEC equipment; increasing the KAP of the population on HIV/AIDS, Malaria and TB by 50% of its 2005 status; and to increase adolescent awareness and knowledge on HIV/AIDS and Sexually Transmitted Infections from 77% and 30% to 95% and 80%, respectively

6. Health Management Information System and Monitoring and Evaluation:- The HMIS and M&E component aims at informed policy formulation, planning, program implementation, monitoring and evaluation and at improving the knowledge and skills of health managers in these areas. It also aims at enhancing community involvement in the management of health facilities and public health interventions. The objectives of this component, is thus to implement the Civil Service Reform Program in the health sector to ensure efficient, effective, transparent, accountable and ethical service delivery at all levels of the health system; to develop and implement a comprehensive and standardized national HMIS and M& E System so as to ensure evidence based planning and management of health services; and to harmonize the donor-government efforts in planning, reporting, monitoring and evaluation.

7. Health Care Financing:- The health care financing component aims at mobilizing increased resources to the health sector; promoting the efficient allocation of resources and developing a sustainable health care financing system. The targets set under this component are increasing overall health expenditure per capita from 5.6 USD to 9.6 USD; doubling the share of health as a proportion of total Government budget; and expanding the Hospital based of Special Pharmacies from 82% to 100% and that of Health Centers from 58% to 100%. In addition social health insurance will be designed and implemented for employees in the formal sector and community health insurance will be designed and pilot tested.

6. Summary

Aspect	HSDP I (1997–2002)	HSDP II (2002–2005)	HSDP III (2005–2010)	HSDP IV (2010–2015)
Main Goal	Restructure and expand basic health services	Expand coverage & strengthen health system	Improve access and equity through decentralization	Improve quality, efficiency & equity of services
Focus Areas	Infrastructure, human resources, essential drugs	Health service coverage, quality, reform	Decentralization, Health Extension Program	MDG targets, quality of care, health financing
Planning Approach	Centralized planning	Begin woreda involvement	Woreda-based planning fully implemented	Strengthen decentralized, results-based planning
Health Extension Program	Not yet introduced	Introduced in late phase	Scaled up nationwide	Consolidated and strengthened
Human Resource Development	Initial training and recruitment	Training schools expanded	Deployment to remote areas	Task-shifting, incentives, retention strategies
Health Infrastructure	Health center construction begins	Increased health posts and centers	Massive expansion of health posts (HEP)	Improved facility functionality and equipment
Community Participation	Minimal involvement	Began involving communities	Health Development Army introduced	Expanded HDA and women's development groups
Equity Emphasis	Low	Moderate focus	Strong focus on rural and pastoralist areas	Strengthened equity in quality and access
Monitoring & Evaluation	Weak M&E system	Improved indicators tracking	Integrated HMIS, supervision tools	Results-based monitoring, Performance reviews
Key Achievements	Laid health system foundation	Improved coverage & immunization	Expanded primary care, improved MCH indicators	Achieved most MDGs, improved quality & equity
Challenges Faced	Weak system, limited funding, poor access	HR shortages, geographic inequities	Sustainability, quality gaps	Health financing, urban service gaps, HR turnover

7.Reference

- Federal ministry of health , health sector strategic plan 2005/6 2009/10
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